A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

14 May 2026

Dr Glen Guerra
Melbourne Colorectal Specialists

Dear Dr Guerra,

Re: Mr Samad Raza
DOB: 20/03/1994 Phone: 0450 806 386
Address: 46 Landsdown Street, BALWYN NORTH, VIC, 3104
Email: samad.raza94@outlook.com

Summary

- **Some improvement in pain severity and frequency over the last eight weeks.**
- **Central sensitisation and hypervigilance regarding bowel movements contributing to ongoing pelvic floor tension.**
- **For consideration of psychological therapy.**
- **No action required by you.**

I had the pleasure of conducting a review with Samad. You will be pleased to know that he has experienced a sustained period of clinical improvement over the last six to eight weeks. He reports that his pain is notably less frequent and less severe, and his "rebound" or recovery time following bowel movements has improved.

While he is not yet symptom-free, I believe we are moving in the right direction. We discussed the physiological drivers of his condition, which I conceptualise as a combination of pelvic floor dyssynergia and visceral hypersensitivity. I do not believe a generic diagnosis of irritable bowel syndrome adequately describes his presentation. The previous Botox treatment provided by you appears to have been beneficial in reducing the immediate symptoms and allowing for a degree of desensitisation to occur.

The primary challenge remains the feedback loop created by central sensitisation and hypervigilance. Samad remains understandably focused on the threat of further mucosal injury or anal tears. This level of threat perception likely amplifies even trivial mucosal injuries and perpetuates pelvic floor hypertonicity. I have been quite explicit with him that we must address these psychological and behavioural factors to ensure a long-term recovery and mitigate the risk of relapse. We discussed the need to "de-amplify" the system to move past this cycle of chronic pain.

Impression and Plan

Samad is scheduled to see you this Friday to discuss the potential for further Botox treatment. From my perspective, the goal of further intervention would be to maintain this window of symptom control to allow further desensitisation of the pain pathways. I have strongly encouraged Samad to re-engage with his psychologist, or a new psychiatrist, to work on the threat perception associated with his bowel habit. We discussed various psychiatric referral options, including Daniel Hubik and specialists at the St Vincent's functional gut clinic who have a cognitive behavioural approach to these complex cases. For the time being,

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Samad prefers to continue with his existing psychologist. I have also recommended that he return to see his pelvic floor physiotherapist, Annabelle, once he has achieved a further period of clinical stability. I have asked him to review with me again in three months' time.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Ms Annabelle Citroen

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